



Clearway
Health

CASE STUDY

BOSTON MEDICAL CENTER

Boston Medical Center Health System Improves Care, Creates Revenue with Specialty Pharmacy Program

Boston's safety net hospital built a specialty pharmacy program to improve access to care, drive increased revenue and reinvest into community programs, such as the country's first hospital-run preventive food pantry and a rooftop garden at Boston Medical Center.



OVERVIEW

Boston Medical Center (BMC), which is part of the Boston Medical Center Health System (BMCHS), is a 514-bed academic medical center driven by a commitment to care for all people, regardless of their ability to pay, and to provide programs and services that enhance overall health in their community. More than half of their patients are from underserved populations and many rely on Medicaid and Medicare. The system’s specialty pharmacies now serve more than 77,000 patients annually.

For several years in a row, BMCHS was experiencing potentially detrimental operating losses. In 2014, the time was right for the system to create a specialty pharmacy program that would not only give back to the community, but would recharge their financial situation.

As a safety net and disproportionate share hospital, BMC’s 340B eligibility provides the opportunity to stretch limited financial resources to better serve their community. The 340B Drug Pricing Program is a U.S. government program that requires drug manufacturers to provide outpatient drugs to eligible healthcare organizations at significantly reduced prices.

“Transformative therapies were emerging and more drugs were eligible for 340B. We saw an opportunity to achieve a change in our financial performance,” said Alastair Bell, President and CEO, Boston Medical Center Health System.

Boston Medical Center Health System (BMCHS) Specialty Pharmacy Footprint

Annual Prescription Volume

On-site pharmacy fills	825,000
Mail order pharmacy fills	350,000
Off-site pharmacy fills	255,000
TOTAL	1,430,000



APPROACH

BMCHS sought to change their pharmacy operations to better integrate with care teams.

Securing specialty medications for patients was previously a manual process. “For providers, higher cost medicines were resulting in significant administrative burdens from payers so we saw an opportunity to support our providers in higher engagement with our patients,” said David Twitchell, Vice President and Chief Pharmacy Officer, Boston Medical Center Health System.

They developed a revenue-generating specialty pharmacy program based on a liaison model enabling pharmacists to partner with providers, nurses, care managers and other care staff.

Operational improvements

The specialty pharmacy program at BMCHS, which is now managed using the same approach and methodology started by Clearway Health, includes the following operational improvements:

- **Operational reporting:** automatic data-fed dashboards to track enrollment and retention of patients; allows near real-time insight to allocate staff appropriately and pivot strategies.

- **Closed-loop inventory process:** practicing “just in time” ordering to avoid margin discrepancies; led by inventory liaisons, allowing for easier data verification and accurate reflection of inventory growth.
- **Financial reporting:** utilizing a pharmacy profit and loss cost management methodology, the system provides an accurate view to forecast revenue.
- **Embedded specialty pharmacy staff:** the Clearway Health team is embedded in BMCHS’s facilities and works alongside providers and care teams to support patients and handle administrative and counseling functions.



RESULTS

Clinical and patient outcomes



Patient Assistance Program

Nearly \$30 million in patient assistance funding has been secured to date. This has helped patients to fill vital prescriptions and avoid having to bear the full expense themselves.



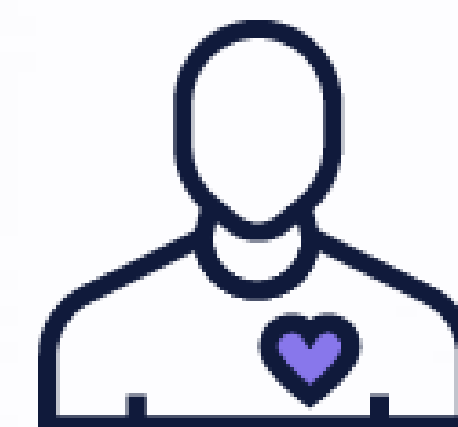
Prior Authorization Time

Turnaround time can be weeks for a new medication pre-authorization, but BMCHS's time is now down to an average of two days.



Medication Adherence

A typical medication adherence rate – Proportion of Days Covered (PDC) – in the industry is 60%. BMCHS has achieved a 95% PDC rate as the result of this specialty pharmacy program.



Chronic Myeloid Leukemia (CML)

CML patients who enrolled in BMCHS's oncology specialty pharmacy program consistently have superior adherence than those who did not enroll. This has led to a 76% major molecular response of those enrolled patients at year two.



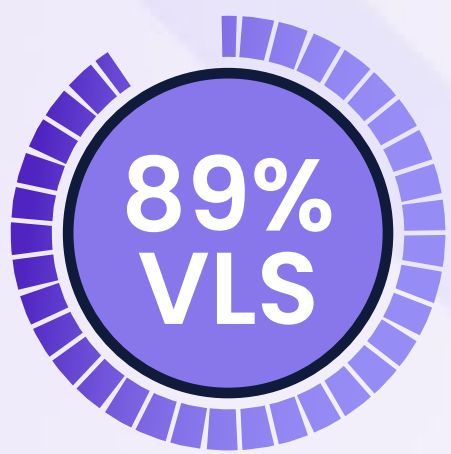
RESULTS

Clinical and patient outcomes



Inflammatory Bowel Disease (IBD)

The PDC for IBD patients enrolled in the specialty pharmacy program is 96% as compared to 88% PDC for those not enrolled. Clinical outcomes also improved, as patients enrolled into BMCHS’s IBD specialty pharmacy program had a more drastic change in average Harvey Bradshaw Index (improvement in disease severity) score over 12 months.



Human Immunodeficiency Virus (HIV)

Because care management is critical to ensure that HIV patients have access to nutrition services through a single point of contact, BMCHS established relationships between the pharmacy and care management teams. This increased patient focus coupled with specialty medication access has led to an 89% viral load suppression for newly diagnosed HIV patients.



Hepatitis C

To provide a more holistic approach to help hepatitis C patients, BMCHS now integrates a social worker with behavioral and substance abuse experience into the pharmacy care team. This innovation, in addition to the embedded specialty pharmacy liaison program, has led to an impressive SVR12 outcome of 90%. SVR, or Sustained Virologic Response, is when a blood test shows no detectable amount of the virus after 12 weeks post-treatment.



FINANCIAL IMPROVEMENTS

Specialty pharmacy revenue has made a significant contribution to Boston Medical Center's ability to improve operations and create new patient care initiatives. From 2012 to 2022, the program grew from \$20 million to \$600 million.

"As an organization, we saw an opportunity to really achieve a step up in financial performance with specialty pharmacy," said Bell. "We went from focusing on capturing large volumes of prescriptions onto a much more focused set of programs targeted at specific patients, specific doctors and specific therapies."

This growth of 30x revenue in a decade can be attributed to many factors, but mainly capturing internal prescriptions instead of relying on contract pharmacies. In addition, BMCHS:

- Took on specialty prescriptions for health plan members in Massachusetts and negotiated with the PBM to take over specialty fulfillment for all members
- Leveraged a centralized enclosed pharmacy program for Medicaid ACO covered lives
- Expanded access to key specialties like select oncology practices, MS, and other specialties to expand patient access and increase revenue opportunity

- Improved LDD access as a result of strategic clinical programming
- Negotiated payer contracts to include in-network specialty pharmacy for over 90% of system volume

Specialty pharmacy partner

Based on their knowledge building their own successful program, BMCHS founded Clearway Health to partner with other hospitals and health systems across the country to build, develop and operate their own specialty pharmacy programs.

Patients benefit from Clearway Health's high-touch engagement, regardless if their course of treatment ranges from a few months or ongoing disease maintenance such as hepatitis C or HIV.

Now, hospitals and health systems who choose Clearway Health to build, strengthen and manage their specialty pharmacy programs benefit from having a partner who has lived experience and delivers rapid outcomes. In 10 years, BMCHS grew specialty pharmacy revenue by more than \$580 million. The speed with which Clearway Health meets and exceeds the revenue opportunity for each of their hospital and health system clients is unmatched – all while putting patients first.





Dr. Alastair Bell
Boston Medical Center
Health System President
and CEO

“We were looking at the rise of specialty drugs and we saw distinct opportunities across patient outcomes, provider experience and financial improvement for our health system. On the patient side, these transformative therapies had potential to cure (such as with hepatitis C). We needed to work much harder to support patients in accessing those medications.”



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